

Government of India

Health Schemes — Eligibility, Application & Requirements Dataset

A Detailed Reference Compendium for Research & Dataset Use

Covering: Ayushman Bharat PM-JAY · National Health Mission · PM-ABHIM · PMSSY · Ayushman Bharat Digital Mission (ABDM) · National AIDS & STD Control Programme (NACP) · Central Government Health Scheme (CGHS) · Pradhan Mantri Bhartiya Janaushadhi Pariyojana (PMBJP) · Free Drugs & Diagnostics Service Initiative · Blood Transfusion Services (NACO)

Compiled from official Government of India websites, ministries, National Health Authority (NHA), National AIDS Control Organisation (NACO), and Press Information Bureau (PIB) releases.

Reference document — August 2026

About This Document

This compendium has been prepared as a structured reference dataset covering ten major Government of India health schemes and programmes. For each scheme it presents: the implementing ministry/nodal agency, launch date, official website, an overview, detailed eligibility criteria, the step-by-step application/access process, documents and requirements, and the official government link for verification.

Content has been drawn from officially published material on Government of India websites (pmjay.gov.in, nhm.gov.in, abdm.gov.in, naco.gov.in, cghs.gov.in, janaushadhi.gov.in, pmabhim.mohfw.gov.in, pmssy.mohfw.gov.in), National Health Authority (NHA) and National AIDS Control Organisation (NACO) guidance documents, and Press Information Bureau (PIB) releases, cross-checked against government scheme-guideline PDFs and the National Government Services Portal (services.india.gov.in) where individual enrolment procedures apply.

Four of the ten schemes covered here — the National Health Mission (NHM), PM-ABHIM, PMSSY, and the Free Drugs & Diagnostics Service Initiative — are systemic, supply-side / infrastructure-strengthening programmes rather than schemes with an individual beneficiary "application form." For these, the "How to Access / Apply" sections describe how citizens actually avail of the services created under the programme (e.g., visiting a public health facility), rather than an individual online enrolment process, since no such individual application exists for these programmes.

Eligibility criteria, fee amounts, document lists, and procedural steps are subject to periodic revision by the respective ministries and implementing agencies. This document is intended for general research, educational, and dataset-compilation purposes. Before taking any action — applying for a scheme, submitting documents, or relying on a specific figure — readers should verify current details directly on the official website listed for that scheme, or at pib.gov.in.

Table of Contents

1. Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB PM-JAY)
 2. National Health Mission (NHM)
 3. Pradhan Mantri Ayushman Bharat Health Infrastructure Mission (PM-ABHIM)
 4. Pradhan Mantri Swasthya Suraksha Yojana (PMSSY)
 5. Ayushman Bharat Digital Mission (ABDM) / ABHA
 6. National AIDS & STD Control Programme (NACP)
 7. Central Government Health Scheme (CGHS)
 8. Pradhan Mantri Bhartiya Janaushadhi Pariyojana (PMBJP)
 9. Free Drugs & Diagnostics Service Initiative
 10. Blood Transfusion Services (under NACO)
- Consolidated Eligibility & Application Quick-Reference Table
- Consolidated List of Official Websites & Helplines

1. Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB PM-JAY)

World's largest government-funded health assurance scheme

Ministry / Nodal Body	Ministry of Health and Family Welfare (MoHFW); implemented by the National Health Authority (NHA)
Launched	23 September 2018, Ranchi, Jharkhand
Official Website	www.pmjay.gov.in nha.gov.in/PM-JAY
Helpline	14555 / 1800-111-565 (toll-free)

Overview

AB PM-JAY provides free, cashless access to healthcare services for economically vulnerable families, offering a health cover of Rs 5 lakh per family per year for secondary and tertiary hospitalisation at empanelled public and private hospitals, with no cap on family size, age, or gender of members.

Eligibility Criteria

Beneficiary families are identified primarily through the deprivation and occupational criteria of the Socio-Economic Caste Census (SECC) 2011. A household is eligible if it meets at least one of the criteria below.

- **Rural households (any one criterion):** living in a single room with kutchha walls and roof; no adult member aged 16–59 years; no adult male member aged 16–59 years; headed by a person with disability and no able-bodied adult member; belonging to Scheduled Castes/Scheduled Tribes; landless households deriving a major part of income from manual casual labour; households without shelter, destitute/living on alms, manual scavenger families, primitive tribal groups, or legally released bonded labourers.
- **Urban households:** workers in defined occupational categories such as street vendors, domestic workers, construction labourers, rag-pickers, cobblers, plumbers, masons, painters, welders, security guards, and other similar informal-sector head-load workers.
- **Senior citizens (2024 expansion):** all citizens aged 70 years and above are eligible for an additional top-up cover of Rs 5 lakh per year, regardless of income or existing socio-economic category; age is determined from Aadhaar records, and Aadhaar-based e-KYC is mandatory for this cohort.
- There is no fixed income ceiling for the core scheme — eligibility is based on living conditions, occupation, and deprivation criteria rather than a stated income limit.
- Beneficiaries already covered under another central/state government health insurance scheme are generally given a one-time option to choose between their existing scheme and AB PM-JAY, to avoid duplicate coverage.

How to Check Eligibility & Apply / Enrol

- **Step 1 — Check eligibility online:** Visit www.pmjay.gov.in and click 'Am I Eligible'. Verify using your registered mobile number and OTP, then search by Aadhaar number, ration card number, or SECC 2011 data along with State and District details.
- **Step 2 — Check via mobile app:** Download the official PM-JAY mobile application (Android/iOS) and use the eligibility-check feature with mobile number verification.

- **Step 3 — Check offline:** Visit the nearest Common Service Centre (CSC), an empanelled hospital's Ayushman Mitra help-desk, or call the 14555 toll-free helpline.
- **Step 4 — Card issuance:** On confirmation of eligibility, an Ayushman Card (e-card) is generated; it can be downloaded from the portal/app or printed at a CSC/empanelled hospital.
- **Step 5 — Availing treatment:** Present the Ayushman Card and a valid ID proof (Aadhaar) at any empanelled hospital's Ayushman Mitra desk to access cashless treatment without upfront payment.
- **Senior citizens (70+):** Aadhaar-based e-KYC is the only requirement; a separate senior-citizen Ayushman Card (Vay Vandana Card) is issued in addition to any existing family card.

Documents / Requirements

- Aadhaar card (mandatory for e-KYC)
- Active mobile number for OTP verification
- Ration card or SECC 2011 household reference (where applicable)
- Proof of category, if claiming under an occupational/urban-worker criterion
- Passport-size photograph (for physical card printing at a CSC/hospital desk)

Coverage & Benefit Details

- Health cover of Rs 5 lakh per family per year for secondary and tertiary hospital care, covering 1,900+ medical/surgical procedures across cardiology, oncology, orthopaedics, and neurosurgery.
- All pre-existing diseases are covered from day one — no waiting period, unlike most private insurance products.
- Coverage includes pre-hospitalisation (up to 3 days) and post-hospitalisation (up to 15 days) expenses.
- Portability: beneficiaries may avail treatment at any empanelled hospital anywhere in India, not only their home state.
- No premium is payable by beneficiaries; the Central and State Governments fund claims directly with empanelled hospitals.

Processing Time & Common Issues

- E-KYC-based eligibility confirmation and Ayushman Card generation is typically instant to a few minutes once identity is verified via Aadhaar OTP.
- Delays commonly arise from mismatched Aadhaar/ration-card details, incorrect mobile-number linkage, or the family name not appearing in the SECC 2011 database — such cases can be escalated through the 14555 helpline or the district Ayushman Bharat nodal officer.
- For the senior-citizen (70+) top-up, only Aadhaar-based age verification is needed; no separate income or SECC check applies to this cohort.

Frequently Asked Questions

- **Is PM-JAY the same as a private health insurance policy?** No — it is a government-funded health assurance scheme; beneficiaries pay no premium, and claims are settled by the Centre and State Governments directly with empanelled hospitals.
- **Can a beneficiary use PM-JAY outside their home state?** Yes, PM-JAY offers full portability, subject to the treating hospital's empanelment status.
- **Does PM-JAY cover pre-existing illnesses?** Yes, from the very first day of enrolment, unlike many private insurance products with waiting periods.
- **Is there an income limit to qualify?** There is no single fixed income ceiling; eligibility is determined by SECC 2011 deprivation/occupational criteria (for the core scheme) or purely by age for the 70+ top-up cover.

Grievance Redressal

- File online via the Central Grievance Redressal Management System (CGRMS) at cgrms.nha.gov.in, quoting the Ayushman/Vay Vandana card number and hospital name.
- Alternatively, file via the central CPGRAMS portal (pgportal.gov.in), which routes the complaint to the National Health Authority.
- Escalation path: District Grievance Redressal Committee, then State Grievance Nodal Officer/State Health Agency, then National Health Authority — grievances are generally required to be addressed within 15–30 days at each tier.
- Call the 14555 toll-free helpline for immediate assistance or complaint registration.

Source: www.pmjay.gov.in | nha.gov.in/PM-JAY; Ministry of Health and Family Welfare / PIB, Government of India.

2. National Health Mission (NHM)

Umbrella programme for strengthening India's public health system

Ministry / Nodal Body	Ministry of Health and Family Welfare (MoHFW)
Launched	NRHM launched 12 April 2005; NHM formed 2013 by subsuming NRHM and NUHM
Official Website	nhm.gov.in

Overview

NHM is India's overarching public health financing and delivery platform, formed in 2013 by bringing the National Rural Health Mission (NRHM, 2005) and the National Urban Health Mission (NUHM) under one umbrella. It funds sub-centres, Primary Health Centres (PHCs), Community Health Centres (CHCs), and district hospitals nationwide.

Nature of the Scheme: No Individual 'Application'

NHM is a financing and health-systems-strengthening framework, not a benefit that an individual applies for. There is no personal eligibility form or beneficiary card under NHM itself; instead, individuals access the services it funds — free maternal care, immunisation, TB/malaria treatment, and NCD screening — directly at public health facilities.

Who Can Access NHM-Funded Services

- Any Indian resident is entitled to use NHM-funded public health facilities (sub-centres, PHCs, CHCs, district hospitals) — there is no income or category restriction for basic outpatient and preventive services at these facilities.
- Pregnant women and children are specifically targeted under the RMNCH+A (Reproductive, Maternal, Newborn, Child and Adolescent Health) component for free antenatal care, institutional delivery, and immunisation.
- Persons requiring treatment for communicable diseases (TB, malaria, kala-azar, filariasis) or non-communicable diseases (diabetes, cardiovascular disease, cancer screening) can access NHM disease-control programmes at any public facility.
- Community-level access is facilitated by Accredited Social Health Activists (ASHAs), who link households to the formal health system.

How to Access Services

- Visit the nearest Sub-Centre, Primary Health Centre (PHC), or Community Health Centre (CHC) — no prior registration or application is required for walk-in outpatient services.
- For immunisation, register at the nearest Anganwadi/PHC or through the U-WIN digital immunisation-tracking platform.
- For maternal health benefits (e.g., Janani Suraksha Yojana cash assistance for institutional delivery, run under NHM), registration is done at the point of antenatal check-up at a government facility, through the ASHA worker or ANM (Auxiliary Nurse Midwife).
- State-specific NHM services and helpline numbers can be found via the State Health Society listed on nhm.gov.in.

Documents / Requirements

- Basic identity proof (Aadhaar or any government ID) for record-keeping at the facility
- Mother and Child Protection (MCP) card for maternal/child health services
- No specific eligibility documents are required to access general OPD or preventive services at a public facility

Institutional Structure & Funding

NHM is steered nationally by the Mission Steering Group and monitored by National Level Monitors, with implementation support from the National Health Systems Resource Centre (NHSRC). At the state level, State Health Societies and District Health Societies drive planning through annual State Programme Implementation Plans (PIPs).

NHM operates on a central-state cost-sharing model, commonly 60:40 between the Centre and general-category States, with a higher Central share for North-Eastern and special-category States and Union Territories.

Key Programmatic Components

- Health Systems Strengthening — infrastructure, human resources, and governance
- Reproductive, Maternal, Newborn, Child and Adolescent Health (RMNCH+A)
- Control of Communicable Diseases (TB, malaria, kala-azar, filariasis, etc.)
- Control of Non-Communicable Diseases (diabetes, cardiovascular disease, cancer)
- Free Drugs and Free Diagnostics Service Initiatives (see Scheme 9)
- Mission Indradhanush for accelerated immunisation coverage

Frequently Asked Questions

- **Is NHM a single scheme or a group of programmes?** NHM is an umbrella framework financing and coordinating multiple sub-missions and disease-control/health-systems components, not a single standalone scheme.
- **Who funds NHM — the Centre or the States?** Both, via a cost-sharing formula, commonly 60:40 between the Centre and general-category States, with a higher Central share for North-Eastern and special-category States/UTs.
- **Does a citizen need to register with NHM to get treated at a PHC?** No — public health facilities funded under NHM are open to walk-in patients; only specific programme components (e.g., maternal-benefit cash schemes) require point-of-care registration through the ASHA/ANM.

Source: nhm.gov.in; Ministry of Health and Family Welfare / PIB, Government of India.

3. Pradhan Mantri Ayushman Bharat Health Infrastructure Mission (PM-ABHIM)

India's largest pan-India scheme for strengthening health infrastructure

Ministry / Nodal Body	Ministry of Health and Family Welfare (MoHFW), implemented in convergence with NHM
Announced / Launched	Announced 1 Feb 2021; Cabinet-approved 15 Sept 2021; launched 25 Oct 2021
Official Website	pmabhim.mohfw.gov.in (also pib.gov.in, dhr.gov.in)

Overview

PM-ABHIM is a Centrally Sponsored Scheme (with Central Sector components) designed to close gaps in health infrastructure exposed by the COVID-19 pandemic, with a total outlay of about Rs 64,180 crore for 2021-26, covering Ayushman Arogya Mandirs, Block Public Health Units, Integrated District Public Health Laboratories, Critical Care Hospital Blocks, and a disease surveillance network.

Nature of the Scheme: No Individual 'Application'

PM-ABHIM funds infrastructure — buildings, laboratories, surveillance systems — implemented by State/District Health Societies. Individuals do not apply to PM-ABHIM directly; they benefit from the facilities it creates, such as Ayushman Arogya Mandirs and Critical Care Hospital Blocks, when visiting a public health facility.

Who Benefits & How Services Are Accessed

- Any citizen visiting an Ayushman Arogya Mandir (upgraded Health & Wellness Centre) can access comprehensive primary care — preventive, promotive, curative, rehabilitative, and palliative services — free of a formal application process.
- Patients requiring critical/ICU-level care at district hospitals in high-priority districts can be treated at PM-ABHIM-funded Critical Care Hospital Blocks on referral or direct admission, as per the hospital's normal admission procedure.
- Diagnostic services are available through Integrated District Public Health Laboratories being strengthened under the mission.
- State/District Health Societies implementing PM-ABHIM projects (for institutions such as NGOs or private partners under PPP components) apply through their respective State Health Department rather than through an individual beneficiary process.

Documents / Requirements (for institutional/PPP participants)

- Registration/incorporation documents of the implementing institution (for PPP components)
- Project proposal aligned with State Programme Implementation Plan (PIP) guidelines
- Compliance documentation per Ministry of Health and Family Welfare and State Health Society norms

Key Components of the Mission

- Upgrading/expanding Ayushman Arogya Mandirs (Health and Wellness Centres) in rural and urban areas

- Establishing Block Public Health Units in high-focus districts
- Strengthening Integrated District Public Health Laboratories in all districts
- Setting up Critical Care Hospital Blocks in high-priority districts (target: 602 districts)
- Building an IT-enabled disease surveillance network at block, district, regional, and national level
- Strengthening health units at Points of Entry (airports, ports, land crossings)

Frequently Asked Questions

- **How is PM-ABHIM different from PM-JAY?** PM-JAY is a demand-side scheme funding treatment costs for eligible patients; PM-ABHIM is a supply-side scheme funding the physical and digital infrastructure — hospitals, labs, surveillance systems — needed to deliver that treatment.
- **What is a Critical Care Hospital Block?** A dedicated critical-care facility, typically attached to a district hospital, built under PM-ABHIM to expand ICU-level treatment capacity.
- **Can an individual apply for PM-ABHIM funding?** No — funding flows to State/District Health Societies and, for select components, to empanelled private partners under PPP norms; there is no individual citizen application.

Source: pmabhim.mohfw.gov.in; pib.gov.in; dhr.gov.in; Ministry of Health and Family Welfare, Government of India.

4. Pradhan Mantri Swasthya Suraksha Yojana (PMSSY)

Scheme for correcting regional imbalances in tertiary healthcare and medical education

Ministry / Nodal Body	Department of Health & Family Welfare, MoHFW
Announced / Approved	Announced 15 August 2003; formally approved March 2006
Official Website	pmssy.mohfw.gov.in

Overview

PMSSY funds the creation of new AIIMS-like institutions and the upgradation of existing government medical colleges to correct regional imbalances in tertiary care and medical education. A total of 22 new AIIMS have been announced under the scheme across successive phases.

Nature of the Scheme: No Individual Beneficiary Application

PMSSY is an infrastructure and institution-building scheme. Citizens do not 'apply' for PMSSY as a personal benefit; instead, they benefit indirectly by accessing OPD/IPD treatment, diagnostics, and specialist care at AIIMS institutions and upgraded medical colleges created under the scheme.

How Patients Access PMSSY-Funded Institutions

- Patients register as outpatients (OPD) at the concerned AIIMS or upgraded medical college through the institution's own registration counter or online OPD appointment portal (where available, e.g., ORS — Online Registration System, ors.gov.in, used across many government hospitals including AIIMS).
- Emergency cases are admitted directly through the casualty/emergency department without prior appointment.
- For postgraduate medical education seats created under PMSSY-funded institutions, admission is through the National Eligibility-cum-Entrance Test (NEET-PG) and centralised counselling conducted by the Medical Counselling Committee (MCC), following normal medical-education admission rules — not a PMSSY-specific application.
- For institutional/contractor participation in construction or upgradation projects, applicants respond to tenders floated by the concerned State Government or the Ministry of Health and Family Welfare through standard public-procurement channels.

Documents / Requirements

- Valid government ID (Aadhaar or equivalent) for OPD/IPD registration at any AIIMS/upgraded institution
- Referral letter, if applicable, from a lower-tier facility for tertiary-care admission
- NEET-PG scorecard and counselling allotment letter for postgraduate medical seats

Two Core Components

- **Component I:** Setting up new All India Institute of Medical Sciences (AIIMS)-like institutions offering tertiary care, medical education, and research.
- **Component II:** Upgradation of identified existing government medical colleges/institutions in various states, with cost shared between the Centre and the concerned State.
- Focus on postgraduate medical education and specialised research capacity, with state-of-the-art modular operation theatres and diagnostic facilities at upgraded institutions.

Phased Rollout

PMSSY has been implemented in multiple phases since 2006 approval. Phase I set up six original AIIMS-like institutions (Bhopal, Bhubaneswar, Jodhpur, Patna, Raipur, and Rishikesh). Subsequent phases progressively added AIIMS institutions in states including West Bengal, Uttar Pradesh, Assam, Bihar, and Jharkhand, taking the total number of announced AIIMS to 22, alongside expanded medical-college upgradation support.

Frequently Asked Questions

- **Are all AIIMS institutions built under PMSSY?** Most newer AIIMS institutions (beyond the original AIIMS New Delhi, which predates PMSSY) have been established under this scheme.
- **Does PMSSY only fund new AIIMS, or also existing colleges?** Both — PMSSY has two components: setting up new AIIMS-like institutions, and upgrading identified existing government medical colleges through Centre-State cost sharing.
- **How does a patient get treated at a PMSSY-funded hospital?** Through the institution's normal OPD registration or emergency-department admission process — there is no separate PMSSY patient-eligibility check.

Source: pmssy.mohfw.gov.in; Ministry of Health and Family Welfare / PIB, Government of India.

5. Ayushman Bharat Digital Mission (ABDM) — ABHA

India's digital health backbone for interoperable, patient-controlled health records

Ministry / Nodal Body	Ministry of Health and Family Welfare; implemented by the National Health Authority (NHA)
Launched	Pilot 15 August 2020; nationwide rollout 27 September 2021
Official Website	abdm.gov.in healthid.ndhm.gov.in (ABHA portal)
Helpline	1800-11-4477 (toll-free) / abdm@nha.gov.in

Overview

ABDM builds interoperable digital health infrastructure through the Ayushman Bharat Health Account (ABHA — a unique 14-digit health ID), the Healthcare Professionals Registry (HPR), the Health Facility Registry (HFR), and consent-based health-information exchange. Over 90 crore ABHA accounts had been created as of 2026.

Eligibility Criteria

ABHA creation is deliberately open and inclusive.

- Any resident of India is eligible to create an ABHA number, regardless of age, gender, income, or health/insurance status.
- A valid Aadhaar number or driving licence is required for identity verification (Aadhaar-based creation is faster).
- A working mobile number capable of receiving an OTP is required for verification.
- Foreign nationals residing in India can also create an ABHA account if they hold a valid identity document and an Indian mobile number.
- ABHA creation for minors/family members can be done by a parent/guardian using their own Aadhaar/mobile number on the member's behalf.
- There is no cost to create an ABHA number or ABHA Card — the service is entirely free.

How to Apply / Create an ABHA Number

- **Online — Aadhaar route:** Visit the official ABHA portal (abdm.gov.in or healthid.ndhm.gov.in) or the ABHA mobile app, select 'Create ABHA Number', choose 'Aadhaar', enter the 12-digit Aadhaar number, verify via OTP sent to the Aadhaar-linked mobile number, complete basic profile details (name, DOB, gender, address, optional email/photo), choose a unique ABHA address (e.g., name@abdm), and submit to generate the ABHA number instantly.
- **Online — Driving Licence route:** Select 'Driving Licence' instead of Aadhaar, enter the licence details, verify with OTP on the registered mobile number, and complete the profile.
- **Offline:** Visit the nearest Ayushman Bharat-empanelled hospital, Common Service Centre (CSC), or government health facility with ID proof, address proof, and a passport-size photograph; facility staff assist with on-the-spot ABHA creation.
- **Download/print the card:** After creation, the ABHA Card can be downloaded as a PDF or printed at a CSC/health facility.
- **Linking health records:** Once created, ABHA is linked to a healthcare provider only after the individual grants explicit, revocable consent through the ABHA app.

Documents / Requirements

- Aadhaar card or driving licence (identity verification)
- Active mobile number (OTP verification); Aadhaar-linked mobile preferred for the Aadhaar route
- Passport-size photograph and address proof (for offline creation at a CSC/facility)

Key Building Blocks of ABDM

- Ayushman Bharat Health Account (ABHA) — unique digital health ID for citizens
- Healthcare Professionals Registry (HPR) — verified registry of doctors and health workers
- Health Facility Registry (HFR) — registry of hospitals, clinics, and diagnostic labs
- Health Information Exchange & Consent Manager (HIE-CM)
- Unified Health Interface (UHI) — enabling appointment booking and teleconsultation
- National Health Claims Exchange (NHCE) — for streamlined insurance claims processing

Consent & Privacy Safeguards

All data-sharing under ABDM is strictly consent-based: healthcare providers can access a patient's linked health records only after the patient explicitly grants consent through the ABHA app or an ABDM-integrated platform, and this consent can be revoked at any time. Newborns are issued a health ID linked to their mother's ABHA number, extending the digital health record framework from birth.

Frequently Asked Questions

- **Is creating an ABHA ID mandatory?** No, it is voluntary for citizens generally, though it is increasingly recommended and, in some contexts such as CGHS, integration with ABHA has been mandated for administrative record-keeping.
- **Is there any cost to create an ABHA number?** No — creating an ABHA number and ABHA Card is completely free.
- **Can I create ABHA without Aadhaar?** Yes, a driving licence can be used as an alternative identity document for verification.

Grievance Redressal

- ABHA-related complaints can be raised through the ABHA Grievance Portal, by writing to abdm@nha.gov.in, or by calling the toll-free number 1800-11-4477.
- General grievances can also be routed through CPGRAMS (pgportal.gov.in) under the Ministry of Health and Family Welfare.

Source: abdm.gov.in; National Health Authority (NHA); Ministry of Health and Family Welfare, Government of India.

6. National AIDS & STD Control Programme (NACP)

India's programme for HIV/AIDS and STI prevention, testing, and treatment

Ministry / Nodal Body	National AIDS Control Organisation (NACO), MoHFW
Launched	NACO established 1992; NACP Phase-V running through FY 2025-26
Official Website	naco.gov.in

Overview

NACO leads India's HIV/AIDS response through a nationwide network of Integrated Counselling and Testing Centres (ICTCs) and Anti-Retroviral Therapy (ART) centres, offering free testing, counselling, and lifelong treatment. The programme operates under the legal backing of the HIV and AIDS (Prevention and Control) Act, 2017.

Eligibility Criteria

- **HIV testing/counselling (ICTC services):** open to any individual, of their own free will or on referral from a medical provider; no restriction by age, income, or citizenship status is applied for basic testing and counselling at a government ICTC.
- **Anti-Retroviral Therapy (ART):** available free of cost to all persons confirmed HIV-positive who need treatment, per national ART initiation guidelines; special emphasis is given to HIV-positive pregnant women and children.
- **Prevention of Parent-to-Child Transmission (PPTCT):** targeted at pregnant women attending antenatal care at government health facilities.
- **Targeted Interventions:** available to high-risk and bridge populations, delivered in partnership with NGOs and community-based organisations.

How to Access Testing & Treatment

- Visit the nearest Integrated Counselling and Testing Centre (ICTC) — typically located in government hospitals, medical colleges, and district hospitals — for free, confidential HIV counselling and testing; no prior appointment or referral letter is required for walk-in testing.
- Locate the nearest ICTC/ART centre through the state AIDS control society or the NACO website (naco.gov.in), or call the state helpline listed on the NACO portal.
- On a confirmed positive diagnosis, the ICTC links the patient to the nearest ART centre for registration and initiation of free antiretroviral treatment.
- Ongoing ART care includes CD4 testing, monitoring, and management of opportunistic infections, provided at no cost at the ART centre.
- Link Antiretroviral Treatment (LAC) and LAC-Plus centres allow patients to collect ARV drugs closer to their residence rather than travelling to the main ART centre.

Documents / Requirements

- No mandatory identity document is required for anonymous/confidential HIV testing at a government ICTC, though a valid ID (Aadhaar or any government ID) is generally requested for ART registration and continuity of care.
- For PPTCT services, the mother's antenatal care (ANC) card/MCP card

Programme Evolution (NACP I–V)

- **NACP-I (1992–1999):** Awareness generation and surveillance systems; established the National AIDS Control Board and NACO; enforced safe-blood and preventive services.
- **NACP-II (1999):** Reduced HIV spread and built institutional response capacity; launched the national Anti-Retroviral Treatment (ART) programme; created the National AIDS Prevention and Control Policy (2002).
- **NACP-III:** Scaled up targeted interventions among high-risk groups, integrated with Care, Support & Treatment (CST) services.
- **NACP-IV / Phase-V:** Comprehensive prevention, detection, and treatment package targeting the 90-90-90/95-95-95 global goals; Phase-V is a Central Sector Scheme fully funded by the Government of India with an outlay of approximately Rs 15,471.94 crore, running through FY 2025-26.

Legal Framework

The HIV and AIDS (Prevention and Control) Act, 2017 provides the legal backbone for the programme, prohibiting discrimination against persons living with HIV/AIDS in employment, healthcare, education, and other domains.

Frequently Asked Questions

- **Is treatment under NACP free of cost?** Yes — ART and HIV testing/counselling services at NACO-supported centres are free of cost.
- **What legal protections exist for people living with HIV in India?** The HIV and AIDS (Prevention and Control) Act, 2017 legally prohibits discrimination in employment, education, healthcare, and housing.
- **Do I need a doctor's referral to get tested?** No — ICTC testing is available on a walk-in, voluntary basis as well as on medical referral.

Grievance Redressal & Confidentiality

- Complaints regarding testing, counselling, or ART service quality can be raised with the concerned State AIDS Control Society or through the CPGRAMS portal (pgportal.gov.in) under the Ministry of Health and Family Welfare.
- Strict confidentiality of HIV status is maintained at every stage of testing and treatment, consistent with the HIV and AIDS (Prevention and Control) Act, 2017.

Source: naco.gov.in; Ministry of Health and Family Welfare / PIB, Government of India.

7. Central Government Health Scheme (CGHS)

Comprehensive healthcare scheme for central government employees, pensioners, and dependents

Ministry / Nodal Body	Ministry of Health and Family Welfare
Established	1954
Official Website	cghs.gov.in cghs.nic.in

Overview

CGHS provides comprehensive outpatient and inpatient medical care — including AYUSH systems — to serving and retired central government employees and their eligible dependents, operational in 75+ CGHS-covered cities across India.

Eligibility Criteria

- Serving central government employees drawing pay from central civil estimates, who make the prescribed monthly CGHS contribution.
- Central government pensioners and eligible employees/pensioners of certain autonomous and statutory bodies, via a one-time or annual contribution.
- Certain other categories including ex-Governors, ex-Lt. Governors, sitting and former Members of Parliament, and freedom fighters, as notified.
- Eligible dependent family members: spouse; dependent parents; unmarried daughters (until marriage or employment, irrespective of age); unmarried, unemployed sons (generally up to age 25, or beyond if pursuing full-time education and unemployed, or indefinitely if disabled).
- The beneficiary and dependents must reside in — or the beneficiary must be posted in — a CGHS-covered city for direct access to CGHS Wellness Centres.

How to Apply

- **Step 1:** Visit the official CGHS portal (cghs.gov.in / cghs.nic.in) and select 'Apply for Plastic Card' (serving employees) or the pensioner application option.
- **Step 2:** Fill in the online application with personal, service/pension, and dependent details (name, department/PPO number, designation, pay level).
- **Step 3:** Upload scanned photographs, signatures, and proof-of-dependency documents for each family member.
- **Step 4:** Submit the form online and print the filled application.
- **Step 5 (serving employees):** Forward the printed application and documents to your administrative department (Head of Office) for verification and endorsement before it is sent to the CGHS office.
- **Step 5 (pensioners):** Submit the printed application along with attested copies of the Pension Payment Order (PPO)/Provisional PPO or Last Pay Certificate, and a demand draft for the prescribed contribution payable to 'PAO CGHS New Delhi', directly to the CGHS Wellness Centre.
- **Step 6:** On verification, CGHS issues the plastic beneficiary card; the card can also be applied for in advance by a serving employee for use after retirement.
- **Renewal (pensioners):** Annual subscription, where applicable, is payable by 31 March; lapsed cards can be reactivated on payment of arrears.

Documents / Requirements

- Duly filled CGHS application form (online, printed and signed)
- Proof of residence of the applicant and dependents
- Age proof for dependent children; disability certificate from a competent authority, if applicable
- For pensioners: attested copies of PPO/Provisional PPO/Last Pay Certificate, and a demand draft for the prescribed contribution
- Passport-size photographs of the applicant and all dependents
- ABHA number linkage (mandated from 1 April 2024) for integration with the digital health ecosystem

Key Benefits

- Outpatient (OPD) consultations, diagnostic tests, and free prescribed medicines
- Cashless inpatient hospitalisation at CGHS Wellness Centres, government hospitals, and empanelled private hospitals
- Coverage for AYUSH systems — Ayurveda, Yoga, Unani, Siddha, and Homoeopathy
- Maternity and child healthcare services
- Emergency medical care and reimbursement for approved emergency treatment

Ward Entitlement, Card Validity & Continuity

Hospital ward entitlement (general, semi-private, or private) is determined by the pay scale/pay level of the beneficiary. A serving employee's CGHS card remains valid until retirement and can be carried over on transfer between departments. On the death of the primary cardholder, dependent family members must apply for a fresh card in their own right to continue availing benefits.

Frequently Asked Questions

- **Can CGHS beneficiaries also hold private health insurance?** Generally yes, though beneficiaries enrolled in certain other public schemes may need to choose between overlapping government schemes as per applicable rules.
- **Is CGHS available in every Indian city?** No — CGHS operates in a defined list of 75+ CGHS-covered cities; those outside these cities are typically covered under alternative arrangements.
- **Can a serving employee apply for a pensioner card in advance?** Yes, an employee currently serving can apply for their pensioner CGHS card while still in service.

Grievance Redressal

- Beneficiaries can lodge complaints — e.g., regarding delayed reimbursement or treatment issues — directly on the CPGRAMS portal (pgportal.gov.in), which routes the grievance to CGHS for resolution.
- Complaints can also be raised in person or in writing at the concerned CGHS Wellness Centre or the Additional Director (CGHS) office for the city.

Source: cghs.gov.in | cghs.nic.in; Ministry of Health and Family Welfare / PIB, Government of India.

8. Pradhan Mantri Bhartiya Janaushadhi Pariyojana (PMBJP)

Nationwide network of stores offering quality generic medicines at affordable prices

Ministry / Nodal Body	Department of Pharmaceuticals, Ministry of Chemicals and Fertilizers; implemented through PMBI (formerly BPPI)
Launched	As 'Jan Aushadhi Scheme', November 2008; rebranded PMBJP in 2016
Official Website	janaushadhi.gov.in

Overview — For Citizens (Buying Medicines)

Any citizen can walk into a Pradhan Mantri Bhartiya Janaushadhi Kendra (PMJAK/PMBJK) and purchase generic medicines and surgical consumables without registration, an eligibility check, or a prescription requirement beyond what is legally needed for the medicine itself. As of January 2026, the scheme offers about 2,110 medicines and 315 surgical items across 29 therapeutic categories, priced typically 50–80% below comparable branded medicines, through more than 11,000 functional Kendras nationwide.

Eligibility Criteria — To Open a Janaushadhi Kendra (PMBJK)

- Any State Government agency, NGO, trust, charitable institution, private hospital, doctor, unemployed pharmacist, or individual entrepreneur is eligible to apply for a new Kendra.
- The applicant must be an Indian citizen, generally at least 18 years old.
- The applicant must hold a Diploma in Pharmacy (D.Pharm) or Bachelor's in Pharmacy (B.Pharm) — or postgraduate (M.Pharm) — degree, or must employ a qualified D.Pharm/B.Pharm-holder as the store pharmacist.
- The applicant must own or lease a minimum of 120 sq. ft. of retail space, supported by a proper lease agreement or space-allotment letter, used solely for operating the Kendra.
- Special incentive categories — entrepreneurs from SC/ST communities, persons with disabilities (Divyaang), and any entrepreneur setting up a Kendra in an NITI Aayog-notified aspirational district, Himalayan/Island territory, or North-Eastern State — are eligible for enhanced financial incentives.

How to Apply to Open a Kendra

- **Step 1:** Visit the official PMBJP registration portal at janaushadhi.gov.in/online_registration.aspx and click 'Apply for Kendra'.
- **Step 2:** Register a new account (or log in if already registered) with basic applicant details.
- **Step 3:** Complete the detailed online application form covering applicant category, proposed location, space details, and pharmacist qualification.
- **Step 4:** Upload the required self-attested documents (see below).
- **Step 5:** Pay the non-refundable application fee (approximately Rs 5,000, subject to revision) via NEFT or other prescribed online payment mode.
- **Step 6:** On scrutiny and approval by PMBI (Pharmaceuticals & Medical Devices Bureau of India), the applicant signs an agreement with PMBI before commencing operations, obtains a drug licence in the Kendra's name, and completes any training arranged by PMBI for first-time retailers.

Documents / Requirements

- Aadhaar card and PAN card of the applicant

- Pharmacy degree certificate (D.Pharm/B.Pharm/M.Pharm) of the applicant or the employed pharmacist
- Proof of space (ownership document or lease/rent agreement, minimum 120 sq. ft.)
- For institutions (NGOs/trusts/hospitals): certificate of incorporation/registration certificate
- Bank account details for incentive disbursement
- Drug licence application (to be obtained in the Kendra's name after approval)

Financial Incentives for Kendra Operators

- Reimbursement/incentive support up to Rs 5 lakh for eligible Kendras (in the form of incentive on monthly purchases, up to 15% of monthly sale value, subject to a monthly ceiling), as per prevailing PMBI norms.
- Enhanced one-time support of about Rs 2 lakh (in addition to normal incentives) for entrepreneurs from SC/ST/Divyaang categories and Kendras opened in NITI Aayog-notified aspirational districts or Himalayan/Island/North-Eastern areas — including furniture and fixture reimbursement and computer/internet equipment support.
- Credit facility for medicine stock is available against post-dated cheques for a defined period (commonly 30 days), as per the PMBI agreement.
- BPPI/PMBI arranges training sessions for entrepreneurs without prior experience in running a medical store.

Frequently Asked Questions

- **Are Janaushadhi generic medicines as effective as branded medicines?** Yes — they contain the same active pharmaceutical ingredients, in the same strength and dosage form, and must meet the same quality standards as branded equivalents, typically at 50–80% lower cost.
- **Who can open a Janaushadhi Kendra?** Individuals, entrepreneurs, and organisations — including special-incentive categories such as women, SC/ST individuals, differently-abled persons, and applicants in North-Eastern, backward, and tribal districts — can apply through the Department of Pharmaceuticals/PMBI to open a Kendra.
- **Does a citizen need any documents to buy medicine at a Kendra?** No formal eligibility documents are required to purchase over-the-counter generic medicines; a valid prescription is required only for prescription-only drugs, as with any pharmacy.

Grievance Redressal

- Complaints regarding Kendra operations, medicine quality, or application status can be raised via the PMBJP customer-care toll-free number, through the janaushadhi.gov.in contact/grievance section, or via CPGRAMS (pgportal.gov.in) under the Department of Pharmaceuticals.

Source: janaushadhi.gov.in; PMBI Guidelines for Opening of New PMBJK; Department of Pharmaceuticals, Government of India.

9. Free Drugs & Diagnostics Service Initiative

NHM initiative to eliminate out-of-pocket spending on essential medicines and diagnostic tests

Ministry / Nodal Body	Ministry of Health and Family Welfare, under the National Health Mission (NHM)
Announced / Launched	Announced in Union Budget 2014-15; guidelines issued 2 July 2015; diagnostics initiative launched 2015
Official Website	nhm.gov.in (Free Drugs & Diagnostics Service Initiative section)

Overview

This twin initiative under NHM provides essential drugs and diagnostic tests — including pathology, radiology, and select advanced tests such as CT scans — free of cost at public health facilities from the sub-centre level up to district hospitals.

Nature of the Scheme: No Individual 'Application'

Like NHM itself, this is a facility-level, policy-driven initiative rather than a scheme with an individual enrolment process. Every patient treated at a participating public health facility is automatically entitled to free essential drugs and diagnostics as prescribed by the attending doctor, without a separate application.

Eligibility & How to Access

- Any patient — resident or otherwise — being treated as an outpatient or inpatient at a public health facility implementing the State's notified Free Drug/Free Diagnostics policy is entitled to receive prescribed essential generic drugs and eligible diagnostic tests free of cost.
- To access the benefit, simply visit the relevant public health facility (sub-centre, PHC, CHC, or district hospital) and consult the attending physician, who prescribes from the facility's Essential Drug List.
- Advanced diagnostics such as CT scans are available (in supported districts, generally those with population above 7.5 lakh) at district hospitals through either an in-house laboratory or a public-private partnership (PPP) arrangement — patients are referred internally by the treating doctor.
- Availability of a specific free drug or test depends on the facility level and the individual State's implementation model (in-house vs. outsourced); patients can confirm availability at the facility's pharmacy or diagnostic counter.

Documents / Requirements

- Standard hospital OPD/IPD registration slip or case-record number issued at the facility
- Doctor's prescription (generated at the point of consultation) — no separate application form is required by the patient

Implementation Approach

- All States/UTs have notified policies to provide free essential drugs at public health facilities.
- Model Request-for-Proposal (RFP) documents and operational guidelines have been issued to states for consistent implementation of the diagnostics component.
- States are supported to set up CT scan and other advanced diagnostic services at district hospitals under public-private partnership (PPP) or in-house models.

- Coverage has been extended to life-threatening conditions such as cancer, thalassemia, haemophilia, and neurological, cardiac, and renal disorders in several states.
- Emphasis on procurement of generic drugs to control costs while maintaining quality, using e-Tendering and two-bid (technical and price) procurement systems.

Frequently Asked Questions

- **Are these tests and drugs free at all public hospitals?** The initiative applies to public health facilities implementing the State's notified Free Drug/Free Diagnostics policy, from sub-centres up to district hospitals; actual availability depends on the facility level and the State's implementation model.
- **Does the Free Diagnostics Initiative include advanced tests like CT scans?** Yes, in supported districts — States have been assisted in setting up CT scan services at district hospitals (population above 7.5 lakh) under PPP or in-house models.

Source: nhm.gov.in (Free Drugs & Diagnostics Service Initiative section); Ministry of Health and Family Welfare / PIB, Government of India.

10. Blood Transfusion Services (under NACO)

National framework for safe, adequate, and voluntary blood supply

Ministry / Nodal Body	National AIDS Control Organisation (NACO), MoHFW; apex body: National Blood Transfusion Council (NBTC)
Established	NBTC constituted 1996; National Blood Policy adopted April 2002
Official Website	naco.gov.in (Blood Transfusion Services) nbtc.naco.gov.in eraktkosh.mohfw.gov.in

Overview

NACO, through the National Blood Transfusion Council (NBTC) and State Blood Transfusion Councils (SBTCs), supports a network of over 1,100 blood banks across more than 600 districts, built on a 100% voluntary, non-remunerated donation model. The e-RaktKosh portal (eraktkosh.mohfw.gov.in) provides a centralised, searchable directory of blood banks and stock availability nationwide.

Eligibility Criteria — To Donate Blood

- Age: generally 18 to 65 years (first-time donors typically accepted up to 60 years).
- Weight: minimum 45 kg body weight.
- Haemoglobin level: minimum 12.5 g/dL, checked at the blood bank before donation.
- Normal pulse, blood pressure, and body temperature at the time of donation.
- Must be free of active infection (flu, cold, sore throat, fever, stomach infection) at the time of donation.
- Deferral periods apply for recent tattoos/piercings (6 months), certain vaccinations (varies by vaccine type), recent childbirth/lactation (donation deferred until roughly one year post-delivery and cessation of breastfeeding), and specific medications or recent surgery.
- Persons who have ever tested positive for HIV, and pregnant or currently breastfeeding women, are not eligible to donate.
- Minimum interval between donations: approximately 3 months (90 days) for men and 4 months (120 days) for women, for whole-blood donation.

Eligibility Criteria — To Request/Receive Blood

- Any patient requiring blood or blood components on medical advice, at any hospital connected to a licensed blood bank, is eligible to receive blood; there is no separate government 'application' — access is through the treating hospital's blood-requisition process, which verifies compatibility and issues the unit against a doctor's requisition.

How to Donate Blood

- **Step 1:** Locate the nearest licensed blood bank or an upcoming blood donation camp via the e-RaktKosh portal (eraktkosh.mohfw.gov.in) or the concerned State Blood Transfusion Council.
- **Step 2:** Register at the centre with a valid photo ID; complete a brief donor questionnaire on health history and risk factors.
- **Step 3:** Undergo a short pre-donation health screening (haemoglobin check, blood pressure, pulse, weight).
- **Step 4:** Donate whole blood (350–450 ml depending on body weight) or, at equipped centres, donate specific components (platelets/plasma) via apheresis; the process including screening typically takes 45 minutes to a few hours depending on the donation type.
- **Step 5:** Rest and refreshment post-donation as advised by blood bank staff.

How to Request Blood (as a Patient/Attendant)

- Obtain a blood requisition slip from the treating doctor/hospital specifying blood group and units required.
- Submit the requisition at the hospital blood bank or an affiliated blood centre; cross-matching is performed before issue.
- Use e-RaktKosh to check real-time blood-stock availability by blood group at nearby blood banks in an emergency.

Documents / Requirements

- For donors: valid government photo ID (Aadhaar or equivalent)
- For recipients: doctor's blood requisition slip from the treating hospital

National Blood Policy — Core Objectives & Legal Milestones

A 1996 Supreme Court judgment (CWP 91/1992) mandated creation of the National Blood Transfusion Council and the phasing out of professional (paid) blood donation; mandatory licensing of blood centres was enforced from 1997, and professional blood donation was completely banned by 1998. The National Blood Policy of 2002 laid out a comprehensive framework — with eight core objectives — including reiterating government commitment to provide safe and adequate blood, components, and blood products; developing and reorganising Blood Transfusion Services (BTS); and deploying the latest technology in BTS operations.

Network & Scale

- NACO supports a network of over 1,100 blood banks across more than 600 districts, including Model Blood Banks and Blood Component Separation Units (BCSUs).
- State Blood Transfusion Councils (SBTCs) are functional in the majority of States/UTs.
- National annual blood requirement is estimated at roughly 1.28 crore (12.8 million) units, based on the WHO norm of 1% of population.
- Blood centre licensing falls under the Drug Controller General of India, under the Drugs and Cosmetics Act, 1940, while policy oversight rests with the NBTC and SBTCs.

Frequently Asked Questions

- **Is professional (paid) blood donation legal in India?** No — it was completely banned by 1998, and the system now relies entirely on voluntary, non-remunerated donors.
- **Who licenses blood banks in India?** The Drug Controller General of India, under the Drugs and Cosmetics Act, 1940; policy oversight rests with the National and State Blood Transfusion Councils.
- **Is blood donation free, and is receiving blood free?** Donation is voluntary and non-remunerated; blood/component costs to patients (processing charges) vary by hospital and state policy — many public hospitals provide it free or at nominal processing cost.

Grievance Redressal

- Complaints regarding blood bank services, donor treatment, or blood quality can be raised with the concerned State Blood Transfusion Council (SBTC), the National Blood Transfusion Council (nbtco.naco.gov.in), or through CPGRAMS (pgportal.gov.in).

Source: naco.gov.in (Blood Transfusion Services) | nbtco.naco.gov.in | eraktkosh.mohfw.gov.in; Ministry of Health and Family Welfare, Government of India.

Consolidated Eligibility & Application Quick-Reference Table

The table below summarises, at a glance, whether each scheme involves an individual application/enrolment step, and the primary access channel a citizen would use.

AB PM-JAY	Yes — eligibility check + Ayushman Card	ajay.gov.in / CSC / empanelled hospital
NHM	No — walk-in public facility access	Sub-centre / PHC / CHC / district hospital
PM-ABHIM	No (institutional/PPP only)	Ayushman Arogya Mandir / district hospital
PMSSY	No (OPD registration only)	AIIMS / upgraded medical college OPD
ABDM (ABHA)	Yes — ABHA number creation	abdm.gov.in / ABHA app / CSC
NACP	No — walk-in ICTC/ART access	ICTC / ART centre
CGHS	Yes — plastic card application	cghs.gov.in / CGHS Wellness Centre
PMBJP (buying medicines)	No — walk-in purchase	Any Janaushadhi Kendra
PMBJP (opening a Kendra)	Yes — Kendra licence application	janaushadhi.gov.in
Free Drugs & Diagnostics	No — automatic at public facility	Sub-centre to district hospital
Blood Transfusion (donate)	Yes — donor registration at camp/bank	e-RaktKosh / licensed blood bank
Blood Transfusion (receive)	No — doctor's requisition	Hospital blood bank

Consolidated List of Official Websites & Helplines

1	Ayushman Bharat PM-JAY	www.pmjay.gov.in nha.gov.in/PM-JAY 14555
2	National Health Mission (NHM)	nhm.gov.in
3	PM-ABHIM	pmabhim.mohfw.gov.in pib.gov.in dhr.gov.in
4	PMSSY	pmssy.mohfw.gov.in
5	Ayushman Bharat Digital Mission (ABDM)	abdm.gov.in healthid.ndhm.gov.in 1800-11-4477
6	National AIDS & STD Control Programme (NACP)	naco.gov.in
7	Central Government Health Scheme (CGHS)	cghs.gov.in cghs.nic.in
8	PMBJP	janaushadhi.gov.in
9	Free Drugs & Diagnostics Service Initiative	nhm.gov.in (Free Drugs & Diagnostics section)
10	Blood Transfusion Services (NACO)	naco.gov.in nbtc.naco.gov.in eraktkosh.mohfw.gov.in

All figures, eligibility thresholds, fees, and procedural steps in this document reflect the most recent publicly available official information at the time of compilation (August 2026) and are subject to periodic revision. For the latest updates, always refer directly to the official website of the concerned scheme or the Press Information Bureau (pib.gov.in).

Appendix A — Glossary of Common Terms

Key abbreviations and terms used across this compendium, useful for dataset labelling and cross-referencing.

ABDM	Ayushman Bharat Digital Mission — India's digital health infrastructure programme.
ABHA	Ayushman Bharat Health Account — a unique 14-digit voluntary digital health ID.
ART	Anti-Retroviral Therapy — lifelong free treatment for persons living with HIV.
ASHA	Accredited Social Health Activist — community-level health worker under NHM.
BPPI / PMBI	Bureau of Pharma PSUs of India, now Pharmaceuticals & Medical Devices Bureau of India — implements PMBJP.
CGHS	Central Government Health Scheme — healthcare scheme for central government employees/pensioners.
CHC	Community Health Centre — secondary-level public health facility.
CPGRAMS	Centralised Public Grievance Redress and Monitoring System — the Government of India's unified grievance portal.
HFR	Health Facility Registry — ABDM registry of hospitals, clinics, and labs.
HPR	Healthcare Professionals Registry — ABDM registry of verified doctors and health workers.
ICTC	Integrated Counselling and Testing Centre — HIV testing and counselling facility under NACP.
MoHFW	Ministry of Health and Family Welfare, Government of India.
NACO	National AIDS Control Organisation — nodal body for HIV/AIDS control under MoHFW.
NBTC	National Blood Transfusion Council — apex policy body for blood transfusion services.
NHA	National Health Authority — implementing agency for AB PM-JAY and ABDM.
NHM	National Health Mission — India's umbrella public health financing and delivery programme.
PHC	Primary Health Centre — first point of contact with a medical officer in rural areas.
PIP	Programme Implementation Plan — annual state-level plan under NHM.
PM-ABHIM	Pradhan Mantri Ayushman Bharat Health Infrastructure Mission.
PM-JAY	Pradhan Mantri Jan Arogya Yojana — the insurance-cover component of Ayushman Bharat.
PMBJP	Pradhan Mantri Bhartiya Janaushadhi Pariyojana — generic-medicine retail scheme.

PMSSY	Pradhan Mantri Swasthya Suraksha Yojana — tertiary-care and medical-education infrastructure scheme.
SBTC	State Blood Transfusion Council — state-level counterpart of the NBTC.
SECC 2011	Socio-Economic Caste Census, 2011 — the deprivation/occupational data source used to determine PM-JAY eligibility.
e-RaktKosh	Centralised blood-bank management and stock-availability portal (eraktkosh.mohfw.gov.in).

Appendix B — Master Document Checklist by Scheme

A consolidated checklist of documents most commonly required across the ten schemes, for quick dataset cross-referencing.

AB PM-JAY	Aadhaar; mobile number; ration card/SECC reference; category proof (urban workers)
NHM	Basic ID (Aadhaar); MCP card (maternal/child services)
PM-ABHIM	Institutional registration & project proposal (PPP/NGO participants only)
PMSSY	Government ID for OPD; referral letter; NEET-PG scorecard (for PG seats)
ABDM / ABHA	Aadhaar or driving licence; active mobile number; photo (offline mode)
NACP	None mandatory for testing; ID for ART registration; ANC/MCP card (PPTCT)
CGHS	Application form; residence proof; PPO/Last Pay Certificate (pensioners); photographs
PMBJP (Kendra)	Aadhaar, PAN, pharmacy degree, space-lease proof, bank details
Free Drugs & Diagnostics	OPD/IPD registration slip; doctor's prescription
Blood Transfusion (donor)	Government photo ID